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Standard: Blood Borne Pathogen (BBP) Standard

1. Introduction

GE is committed to providing a safe work environment for all employees and contractors. This Standard is designed to (i) protect GE employees who may be at risk of exposure to BBP due to their work responsibilities and (ii) comply with country and regional specific regulations in which GE employees work. Please note that country, state, provincial, district or local regulations may be more stringent or contain additional requirements and, in such instances, must be followed.

2. Scope and Field of Application

This Standard applies to all individuals at GE fixed workplaces (“Sites”) and services at distributed locations including GE employees and contractors (“Employees”) whose job classification or designated tasks put them at risk for exposure to blood or body fluids. These include but are not limited to physicians, nurses, emergency responders and volunteers for first aid and/or cardiopulmonary resuscitation (CPR) training.

3. Definitions

Biohazard Bags: Heavy gauge plastic bags designed and certified for disposal of non-sharp bio-hazardous waste. These bags clearly display the universal biohazard waste symbol.

Biological Monitoring (BM): The monitoring of biological indicators, and the examination of materials such as blood or urine, to determine potential exposure to chemical substances, and related risk.

Blood borne pathogen: Pathogenic microorganisms that can cause disease in humans. These pathogens include but are not limited to hepatitis B virus (HBV), hepatitis C (HCV) and human immunodeficiency virus (HIV).

Contaminated: The presence, or the reasonably anticipated presence of blood, body fluids, or other potentially infectious materials on an item or surface.

Exposure Incident: A specific eye, mouth, other mucous membrane, non-intact skin (includes dermatitis, hangnail, cuts, abrasions, chafing, etc.), or injected contact with blood or other potentially infectious materials that results from the performance of an employee's duties.

GE Best Practice: Recommendation based on occupational medicine best-evidence, internal and external benchmarking and company best practices.

GE Policy: Rules or mandates promulgated by the GE Board of Directors or Senior Management.

GE Standard: Requirement based on review of the GE Medical Standards Workgroup (MSWG) and approved by a select panel of Corporate Medical, Legal, HR, and EHS managers.

Medical Surveillance Examination (MSE): Medical examination and testing designed to detect and monitor potential health effects from hazardous chemical or physical exposures in the workplace, to enable early treatment and/or other steps to protect employee health.

OHS: GE Health Services/ Medical

Personal Protective Equipment (PPE): Specialized clothing or equipment worn by an employee for protection against a hazard.

Regulated Medical Waste: Liquid or semi-liquid blood or body fluids; used sharps, and other disposable items that have been in contact with blood or body fluids but have not been fully decontaminated.

Regulatory Requirement: Requirement dictated by municipal, state, provincial, federal or other legal statute.

Sharps Container: A plastic or metal, impenetrable container intended for the storage of used or contaminated needles, blades or broken glass.

Source Individual: The individual whose blood or body fluid was the source of the exposure or contamination. Under *Universal Precautions* blood and fluids from these individuals is always considered potentially infectious.

Standard or Routine Precautions (aka Universal Precautions): An approach to infection control. According to the concept of Universal Precautions, all human blood and certain human body fluids are treated as if known to be infectious for HIV, HBV, and other bloodborne pathogens.

3. Policy

The guidance contained in this document is based on [GE Integrity Environmental, Health & Safety Policy](#) that mandates a safe and healthy workplace.

4. Regulatory Requirements

Federal, provincial, state, and/or local governments typically have established regulations requiring training, medical surveillance and biological monitoring (BM) for employees exposed to specific hazardous materials. In most countries, employee participation in the training and medical surveillance/biological monitoring program is a mandatory condition of employment. In the US, Canada, and certain other countries,

participation is voluntary. Even where participation is voluntary, GE must still operate in such a manner as to protect individuals from occupational hazards.

Prevailing medical record privacy regulations should be considered as part of this program. Employee identifiable information should only be recorded in a confidential medical record, unless otherwise required by regulation.

5. Roles and Responsibilities

The Site Employee Health Services/Medical or designated Environmental Health and Safety (EHS) Leader is responsible for meeting all requirements of an Exposure Control Plan, including conducting an annual evaluation to determine who is at risk of being exposed to BBPs, evaluating controls to reduce the chances of being exposed to BBPs, ensuring that those who might be exposed to BBP complete annual BBP training, managing the post-exposure procedure (including necessary root cause analysis) and all recordkeeping.

5.1 Management

- Provide appropriate controls (engineering, administrative, personal protective equipment (PPE) to protect the health of employees, contractors, and visitors.
- Facilitate BBP training

5.2 Environment, Health and Safety (EHS)

- Advise management on potential exposure risk to BPP
- Support management in the provision of appropriate controls to protect the health of employees.
- Support management BPP training
- Perform any testing required to quantify the risks to workers

5.3 Employees

- Understand the risks associated with BBPs and complete any required training
- Follow all standard operating procedures to reduce risk of potential exposure
- Wear required PPE and utilize safe BPP disposal mechanisms
- Report any known or potential BPP exposures to manager and reach out to Health Services for medical guidance.
- Identify any changes in health status that may be due to, or aggravated by, exposure to BBPs

5.4 Health Services

- Provide medical guidance to employees with known or potential BBP exposure
- Evaluate, or make arrangements, for medical evaluation of employee

- Provide evaluation results to employee
- Provide summary report to management
- Work collaboratively with EHS to identify areas where BBP hazard controls should be improved

6. BBP Program Components

6.1 Risk Assessment

GE Health Services and EHS shall do an annual assessment of the site and service operations, and related BBP plan, to determine which populations of employees are at risk for BBP exposure.

6.2 Mitigation or Elimination of BBP Risk

High level strategies and processes to effectively eliminate or reduce the risk of BBP exposure include:

- a. Eliminating or substituting processes intended to minimize exposure to risk
- b. Implementing engineering controls
 - In many locations, such as the U.S., the use of these protective measures is required by regulation. Examples of engineering controls include the use of sharps with engineered protections (such as retracting or self-capping needles); appropriate sharps containers, and appropriately labeled cabinets and storage containers.
- c. Developing workplace administrative controls and policies
 - Universal blood and body fluid precautions will be observed in caring for all emergency victims.
 - All procedures involving blood or other potentially infectious materials will be performed in a manner designed to minimize splashing, spraying or splattering.
 - Wash hands and exposed skin with soap and water immediately after exposure to infectious materials and after taking off gloves or other PPE. Hand washing facilities should be available to employees who may come into contact with blood or other potentially infectious materials.
 - If skin, eyes, or mucous membranes are exposed, they should be washed or flushed with large quantities of water immediately, or as soon as the imminent danger has concluded.
 - Before a victim is transported, it is important to “rope off” areas of contaminated surfaces with yellow caution tape. Ideally, spills of blood or blood containing body fluids, vomit or urine should be cleaned up by using Biohazard Spill Kits according to the directions provided with those Kits. These Kits contain an acceptable biocide solution.

- If a Biohazard Spill Kit is not available, diluted bleach can be used. Follow the directions on the bottle for preparing a diluted bleach solution. If your bottle does not have directions, you can make a bleach solution by mixing 5 tablespoons (1/3 cup) of bleach per gallon of room temperature water or 4 teaspoons of bleach per quart of room temperature water.
- Bleach powder should not be used to create a cleansing solution, unless supervised by an EHS professional.
- Wash hands after cleaning or disinfecting.
- Make a new diluted bleach solution daily. Bleach solutions will not be as effective after being mixed with water for over 24 hours.
- Always follow the manufacturer's instructions for applying the bleach solution to surfaces.
- If instructions are not available, leave the diluted bleach solution on the surface for at least 1 minute before removing or wiping. This is known as the "contact time" for disinfection. The surface should remain visibly wet during the contact time.
- Cleanup of potentially infectious blood or body fluids should only be performed by an employee who has received BBP training.
- Needles should not be recapped. Sharps containers will be used for contaminated sharps (i.e.) metal and/or broken glass) and disposed of according to all applicable regulations for medical waste.
- A licensed medical waste company will be used for disposal of regulated medical waste.
- Avoid eating, drinking or applying cosmetics in potentially contaminated areas (such as on-site medical clinic employee care rooms).

d. Training

- Employees whose job assignments have potential for exposure to BBP, shall receive training and information within 90 days of assignment to the job.
- BBP training and information shall be communicated annually for affected employees. The site Medical or designated EHS Leader shall ensure that this occurs.
- All employees covered under the BBP standard must be educated on the following topics:
 - The hazards of BBP, methods of transmission of BBP and methods of protection.
 - Review of the site BBP exposure control plan and how to obtain a copy.
 - Engineering, PPE and work practice controls designed to eliminate or minimize exposure to BBP.

- Procedure for handling an accidental occupational exposure to blood or body fluids.
- The availability (at no charge to the employee) of post-exposure Hepatitis B vaccinations provided by the employer.
- How recordkeeping will be handled for any workplace exposure incidents.
- Post-exposure evaluation and follow up
- In the US, as per OSHA 1910.1030(g)(2)(vii)(N) and 1910.1030(g)(2)(viii), employees who take BBP training must have an "interactive" experience.

This means that each participant needs to be provided with an opportunity to ask questions of a qualified/knowledgeable person and to get an immediate answer, particularly related to implementation of BBP program as it relates to the workplace of the participant.

- Solutions that would meet this requirement include face-to-face training; Web based training with a t-con; Web based training with a chat feature; straight t-con; Telephone Hot Line provided by approved vendor or any similar format that provides the required interactivity in real time.
- Solutions such as Support Central sites, documented FAQs or response by e mail or any other system that does not provide real time response will not suffice. Note that if the GE site or organization assigns the EHS online course: [GE-EHS-140: Blood-borne Pathogens Awareness for Potentially Exposed Individuals](#), the chapter for US employees instructs user to access the BPP Helpline, for real-time support for questions as they take this course.:
- [BBP Helpline](#) is available 24 hours/day and 7 days/ week at 1-877-440-9220. Upon calling this hotline, you will connect with an Optum Health-Health Coach Nurse who will help with training related questions.
- [Local Public Health Departments or Red Cross Providers may be a source for face-to-face training offerings.](#)

e. Hepatitis B Vaccination

- All employees engaged in activities that might lead to the accidental exposure to blood or other potentially infectious materials will be offered the Hepatitis B vaccine series at the time of assignment.
- Employees who have either been previously vaccinated or have a documented history of hepatitis B are exempt from vaccination.
- Vaccinations will be provided at no cost to the employee.
- All employees who decline the Hepatitis B vaccine will sign the [Hepatitis B Vaccine Declination Form](#)

- The site EHS or Medical Leader will ensure that the vaccine is offered as required.
 - In adults with intact immune systems, protection from Hepatitis B persists for >20 years after vaccination.
 - Boosters are not routinely recommended.
 - Post-vaccination testing for Hepatitis B immunity is not usually required; however, it may be applicable or mandatory for specific healthcare providers, vendor credentialing, regional/state laws, or as determined by Business/Regional Medical Directors.
- f. Providing PPE to those who might be exposed to BBP
- GE shall provide PPE to employees who might reasonably expected to be exposed to BBP
 - PPE plays an important role in protecting GE employees against BBP.
 - Disposable gloves, used one time on one person only, will be worn where it is reasonably anticipated that employees will have hand contact with blood or other potentially infectious materials, non-intact skin or mucous membranes.
 - Examination/surgical (i.e., nitrile gloves), or other fluid-impermeable gloves should be readily available in clinics, automated defibrillator (AED) packs, and first aid kits.
 - Do not use latex gloves due to risk for allergy
 - Employees should remove contaminated gloves carefully and not touch contaminated outer surfaces.
 - Protective eyewear and masks are to be worn when aerosolization, splashes of blood or potentially infectious material is possible.
 - Mucous membranes will be flushed with water immediately or as soon as possible following contact with blood or other potentially infectious materials.

6.3 Post BBP Exposure Procedures

- An exposure incident is defined as blood or other potentially infectious material, entering into the body through accidental injury with contaminated sharps, such as broken needles, contact with open skin from cuts, hang nails, abrasions, acne, dermatitis, or contact with mucous membranes of the mouth, eyes or nose.

- When an employee experiences an exposure incident:
 - Report incident to the site Health Services team and/or EHS Leader who will complete the Exposure Incident Form (see Attachment E - [BBP Program Template with Associated Administrative Records](#)) and take the employee to the Onsite Medical Clinic, the local emergency room or contracted Occupational Health facility to be evaluated by a certified health professional as soon as possible after the exposure occurred.
- During the evaluation the site Health Services or EHS Leader shall ensure that the health professional determines and documents:
 - The route of exposure and the circumstances related to the incident
 - A determination as to whether Post Exposure Prophylaxis (PEP) medications are appropriate.
 - The identification and status of the source individual.
- The site EHS or Health Services Leader will notify the Business OHS/Medical Leader or Regional Health Services Leader of the exposure incident and provide the completed Exposure Incident Form.
- Record of such event and follow-up will be placed in the employee's medical file.

6.4 Recordkeeping

- All administrative records required by this Standard will be maintained by the facility Health Services Team and/or EHS Leader. These records include:
 - Exposure Control Plan documents
 - Hepatitis B Vaccination Documentation and Declination Forms
 - Exposure Incident Forms
 - Training documentation, including identity of the trainer and copies of the training materials
 - All medical records relating to this plan or to a particular exposure incident will be maintained confidentially by the site Health Services team or EHS leader as appropriate.
- [BBP Program Template with Associated Administrative Records](#)
- The Business and Regional Medical Director will be a source for advice and consultation as needed.
- Any BBP exposure situation must be reported to the Business or Regional Medical Director.

7. Management Systems

7.1 Support resources

Adherence to this Guideline is covered in the [EHS Data Reporting and Recording Technical Standard \(TS5016\)](#) as well as through the Cority Electronic Health. Additional information is available at [GE OneEHS injury & Illness Recordkeeping](#)

7.2 Audit requirements

Compliance with this policy is subject to EHS and/or Health Services audit.

Refer to the [CR5007 Compliance Assurance Core Requirement](#)

7.3 Standard renewal process

Per MSWG renewal process, every 3 years

7.4 Deviation process

As research, regulations and industry policies become available; GE will continue to adopt best practices. Deviation requests should be brought to Health Services and/or EHS.

7.5 Training and communications requirements

Implemented by GE businesses and, led by members of Medical Standards Work Group (MSWG) and EHS. Contact your business training lead for specific requirements

7.6 Contact

Medical Standards Workgroup (MSWG), EHS

8. References

CDC Link: [Cleaning and Disinfecting With Bleach | CDC](#)

US OSHA Code of Federal Regulations Title 29, Section 1910.1030 [1910.1030 - Bloodborne pathogens. | Occupational Safety and Health Administration \(osha.gov\)](#)

U.S. Public Health Service PEP Line Quick Guide for Occupational Exposures: [PEP: Post-Exposure Prophylaxis | National Clinician Consultation Center \(ucsf.edu\)](#)

9.0 GE LINKS

[CR5007 Compliance Assurance Core Requirement](#)

[EHS Data Reporting and Recording Technical Standard \(TS5016\)](#)

[BBP Program Template with Associated Administrative Records](#)

[GE Integrity Environmental, Health & Safety Policy](#)

[Hepatitis B Vaccine Declination Form](#)

GE Learning Course: [GE-EHS-140: Bloodborne Pathogens for Potentially Exposed Individuals](#)

Document History

Document Location

This is an on-line document. Paper copies are valid only on the day they are printed. Refer to the author if you are in any doubt about the currency of this document or information contained within it. Document is stored at GE MSWG SC site: [SupportCentral - Occupational Health Policies, Regulations, Guidelines and Recommendations Community \(ge.com\)](#)

Revision History

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1.00	10/01/2011	Version 1.00	Ajay
2.00		Complete Rewrite and Creation	
2.01	08/22/2016	Training Updates	Lee, P.
3.0	08/30/2022	Review and update document with updated links to training and resources	Clay, R, Ladwig, L, Nolan, T, Alfriend, A, Kalia, N., Wertz, T., Cabanos, R.

Approvals

This document requires the following approvals.

Name	Title	Approval Date
MSWG	Approved	8/30/2022
Legal	Approved	12/01/2022
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